

WHY REPORT B IS THE CEILING

WC/2024/227 — *Shepherd v Workers' Compensation Regulator*. Why a properly-built medico-legal causation report is the single highest-leverage document in the appeal — worth more than any filing, forensic find, or settlement move. 9 August 2026.

The claim, stated plainly

The whole case has a ceiling, and Report B is it. Not one document among many — **the** document. The forecast is unambiguous on this: the entire outcome distribution is **bimodal on one event**, the 12 August consultation and the report that comes out of it. A strong, reasoned causation report puts the matter at roughly **55% to win at a contested de novo hearing and ~70% to settle**. A weak one — or one carrying the fatal "not for medico-legal use" footer — drops it to **~30% / ~45%**. Everything else in the appeal is already built. This is not.

"Ceiling" is the exact word. The case cannot rise above the quality of this report. It sets the height of the roof, and nothing below it — no other filing, no better cross-examination plan, no additional witness, no sharper submission — can lift the outcome past the level that Report B fixes. The reason follows.

1. Causation (s 32(1)) is THE gate — and it is the one thing the manner case cannot supply

s 32 is conjunctive. To win, the appellant must clear three independent blocks, and *all three must hold*:

Block	What it decides	Reconciled probability
s 32(1) causation — employment a <i>significant</i> contributing factor to the injury	Did the workplace make him ill at all?	0.80 (range 0.62–0.91)
s 32(5)(a) fails — at least one unreasonable stressor survives (<i>Mahaffey</i>)	Was the conduct unreasonable / not taken reasonably?	0.76
Discipline / credibility — s 32(5)(b) stays shut, adverse material absorbed, self-rep holds	Does the case stay on "conditions", not "perception"?	0.72

Product, disposition-correlated: **P(win) ≈ 0.42**. The tornado is *flat* — each block swings the result by roughly the same ±0.17. No single block carries the case; every one is load-bearing.

But look at what each block *is made of*. Blocks 2 and 3 are **built and are now matters of conduct, not evidence**. The manner case (block 2) is finished: the 7-hour break is admitted (SOFC ¶122(a)), Factor 4 is already found to be unreasonable management action by the Regulator's own delegate, the 44-day non-response across four decision-makers is established almost entirely from MSH's own writing, and the "asserted, not resolved" framing is in hand. The forecast's own instruction is blunt: "*Do not spend more effort on the manner case — it is built.*" Block 3 is discipline — a standing of conduct at the hearing, not a document to be procured.

Block 1 is the only one still open, and it is the only one that requires a document that does not yet exist.

Here is the crux, and it is the reason causation is the gate rather than merely one gate of three. **The manner case proves the workplace did unreasonable things. It does not — cannot — prove those things made him psychiatrically ill.** You can win every manner argument in the matter, establish the pairing, the

denial, the loop, the never-closing, the whole 44-day sequence — and the Regulator can still stand up and say: *granted, but the psychiatric injury was caused by bereavement, a relationship breakdown, job loss, a pre-existing ADHD/anxiety history and two years of litigation stress; employment was not a significant contributing factor*. The unreasonableness of conduct is not the causation of injury. Causation is the **bridge** between "the workplace did these things" and "the workplace made him ill", and that bridge is a **clinical question answerable only on a history**. Cory proves the facts. The manner case proves the conduct was unreasonable. Neither proves the injury was caused by employment. That gap — precisely and only that gap — is what Report B fills. No amount of forensic work on the manner case reaches across it, because it is a different kind of question, answered by a different kind of witness.

And the Regulator has told us exactly where it will push. **SOFC ¶22(f): the roster issue was "not causative of the appellant's injury."** That attack is aimed at Stressor 3 — the March roster — and it lives in the **interval between the 17–18 March 2024 rostering and the pleaded onset of 18 June 2024** (about three months). Report B's job is to connect the March roster to the June onset — to explain *clinically* how a fatigue/rostering stressor in a sequence that continued through April and May contributes to an onset in June. (Note the onset-to-presentation interval is short and favourable, not a gap: the Hawes certificate records first attendance for this injury on **1 July 2024**, about two weeks after onset; the diagnosis and the fuller clinical record build from there. Do not concede a "three-month gap to first presentation" — it is not what the record shows.) The clinical bridge from the March sequence to the June onset is an observation only the doctor can make. It is the load-bearing wall of block 1, and block 1 gates the case.

2. The Review Decision helps — but it is a tailwind, not the gate

The instinct is: the Regulator's own delegate already found this, so causation is settled. It is not, for three reasons, and understanding why is what makes Report B indispensable rather than merely useful.

It is admitted as a document, not as a determination. Review Decision 69983 records Finding B — "*work events were the sole cause*" — and Finding A, that the 17–18 March rostering "*amounted to unreasonable management action*." Both are genuinely powerful, and their **contents are admitted** at Form 24 ¶37. But they are admitted with **relevance reserved** — "*the appeal is a hearing de novo and it is for the Commission to determine*." That is decisive. The appeal right is s 549(1); the de novo character comes from **s 558(1)(c)**, the power to *set aside the decision and substitute another*. On a de novo hearing the Commission decides causation **afresh, on the evidence before it**. The delegate's finding is **persuasive, not binding — and must never be pleaded as an estoppel**. A finding that must be re-made is not a finding you can rest on.

The Regulator is now actively contesting the very thing the delegate found. ¶22(f) is a positive plea that the injury was *not* caused by the March events. The 2024 finding is a static document; it cannot answer a 2026 contest, it cannot be cross-examined, and it cannot adapt to the competing causes the Regulator now holds (from A4). Only a live expert, giving reasoned oral evidence, can meet a live causation attack.

The delegate cannot get in the box. The person who made the finding is not a witness the appellant can call to defend it. Report B's author is. That is the difference between a helpful piece of history and a decisive piece of evidence.

So the Review Decision is a real tailwind — it is why the causation block reconciles as high as 0.80, and it is a large part of the settlement pressure. But it is a floor the appellant stands on, not the ceiling he reaches. The ceiling is Report B.

3. Why a properly-built Report B is decisive where A4 is not

The temptation is to treat A4 (the 13 February 2025 Krishnaiah report) as the causation evidence and Wednesday as an update to it. **That is the single most important error to avoid.** A4 is a QSuper income-

protection **capacity** snapshot. It is addressed to a claims manager, it carries "*not for medico-legal use*" on every page in two forms, it was written ~7–8 months after the 18 June 2024 onset, and — fatally for use as causation evidence — **it never asks the s 32(1) question**. Worse, on its face it hands the Regulator ammunition: three competing causes in one sentence ("*relationship breakdown, job loss, and bereavement*"), "*complete incapacity*" eleven days before he returned to work, "*restarting Vyvanse*", and the line that reads as s 32(5)(b) in the report's own words — "*can become consumed by issues he perceives as unjust*." The Regulator already holds all of it (Form 24 ¶¶38–39). A4 answers *capacity* in early 2025. It does not answer *causation* in June 2024, and it was never built to.

A properly-constructed Report B is a different document doing a different job, and every structural feature is a place where A4 is weak and B must be strong:

- **The assumed-facts spine.** The report states in terms: "*I am instructed to assume the facts set out in the statement of Mr Shepherd dated [X] and the documents listed at [Y].*" This separates the two jobs — **Cory proves the workplace facts; the doctor opines on assumed facts**. If the workplace description lives only inside the report, it is hearsay, it is attacked there, and the opinion falls with it. A4 has no such structure; its history is the patient's say-so, and it imports facts the delegate rejected.
- **The s 32(1) question answered in terms** — *whether employment was a significant contributing factor*, never "major", employment need not be the only or main cause, competing causes need not be outweighed (*Adams* [2015] ICQ 1 [23], adopting *Hochen*). A4 never poses it.
- **Competing causes named, placed in time, and reasoned through — not omitted.** This is where reports live or die. The strongest single point: the causes A4 lists sit at *different times*, and the later ones **post-date the injury and therefore cannot have caused it** — job loss is the 8 October 2024 dismissal (four months after onset, and the employer's own reversed act); the separation is later still; litigation post-dates the review decision. A report that names three competing causes and explains why employment remains significant *survives* cross-examination. One that runs a clean baseline and omits them reads as advocacy and is destroyed by a single page.
- **The 26 October 2022 anxiety/ADHD entries addressed**, via the aggravation route (s 32(3)(b): aggravation of a pre-existing condition is itself an injury where employment is a significant contributing factor to the aggravation; s 32(4) limits the injury to the extent of the aggravation's effects). "*To the extent any prior vulnerability is established, the workplace exposure is a significant contributing factor to the aggravation of that condition*" — a sentence that survives whatever the 2022 records turn out to say.
- **Diagnosis with criteria and dates, differentials named and excluded, and a complete sources list** — the most-attacked part of any report; "*you were not given X*" is the first cross-examination question, and a short list is a vulnerability while a complete one is armour.
- **The premorbid traits reasoned about, not retracted.** The "*perceives as unjust*" line is confronted so it is not left as the only clinical word on his personality — a doctor who retracts under instruction is finished as a witness, but a report that reasons that diagnosed MDD with an identified stressor is not displaced by conscientiousness or perfectionism absorbs it.
- **The matter described correctly** — a workers' compensation appeal concerning a **course of management conduct**, not "bullying, harassment" (A4's wrong framing, which lets a cross-examiner say the doctor assessed a different complaint from the one before the Commission).
- **No "not for medico-legal use" footer**, confirmed in writing before the appointment.

That is the difference between a document that survives testing and one that hands the Regulator its case. A4 is diagnosis-and-severity evidence and contemporaneous evidence of the mechanism — that is all it should be asked to do. Report B is the causation instrument, and it is the only causation instrument the appellant will have.

4. It also drives the settlement — where most of the value actually sits

The result is carried by **settlement, not the hearing**. P(compensated overall — settle or win) $\approx$ **74%**; P(win at a contested hearing) $\approx$ **42%**; the gap between them is the negotiated resolutions, $\approx$ **58–62%**. And settlement is a function of the Regulator's litigation risk — which is a function of the evidence it faces.

Assemble what the Regulator's file already contains against it: **its own delegate** found causation ("sole cause") and unreasonable management action; **Dr Hawes** recorded no pre-existing condition and work events as the sole cause; the **7-hour break is admitted by everyone**; and MSH has conceded in writing that there was no fatigue framework at the Switchboard until after the injury and "*no consequential changes to operating procedures*." Now add a **solid, reasoned causation report from the treating psychiatrist**, built on assumed facts, weighing the competing causes and landing on a *significant contributing factor*. That combination is very hard for a rational opponent to take to a hearing.

The numbers move exactly on this: a strong Report B lifts settlement to **~70%**; a weak one drops it to **~45%** — a 25-point swing on one document. This is why Report B is the lever even for the majority of outcomes that never reach a hearing. Pitched together with a **Calderbank #3 and s 558(3) costs teeth** (~4 weeks after the report), it is the *lawful* mechanism for raising the Regulator's cost of not settling — not rhetoric, but a genuine shift in its risk. The report is the thing that makes every other settlement move bite.

5. The multiplier — one report, four jobs

Report B is not a single-purpose document. The same fifteen-to-thirty pages, built once, do at least four separate jobs, which is why its leverage compounds:

1. **The causation gate** (s 32(1)) — block 1, above.
 2. **Quantum and prognosis**. Diagnosis (MDD with anxious distress, DSM-5 296.23), severity anchored in *objective* signs and documented treatment escalation (fluoxetine → increased → quetiapine → Vyvanse — treatment intensity is objective evidence of severity), and a prognosis. Value in any settlement or award is a function of severity and prognosis; this is the document that fixes both.
 3. **The aggravation limb** (s 32(3)(b)/(4)) — the fallback that holds even if a prior vulnerability is established. The report that runs a clean baseline has no fallback and dies on one contested page; the report that reasons through aggravation wins on either footing.
 4. **Capacity, expressed consistently with the ECC** — and this is the trap that makes the multiplier a *risk* as well as a strength. The 3 July 2026 ECC certifies **fit with adjustments**, and the RWA pay claim — currently the best near-term income route — runs on exactly that. If Report B says he is *totally unfit*, it contradicts his own employment position and undermines the pay claim. The two sit together only if the report is precise: capacity for the substantive role **with the identified adjustments**, and any current incapacity addressed as a consequence of the **exclusion and its handling** — not as inherent unfitness. One document, correctly built, carries all four; one loose sentence in it can damage a separate live matter. That is leverage in both directions.
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6. The honest limit — the ceiling has conditions

The ceiling is real, but it is not automatic. Three constraints bound it, and each is a reason the report must be *earned*, not assumed:

It must be reasoned, not asserted. "*Because A, B and C, in my opinion...*" survives; a bare conclusion is worth little and breaks under the first probing question. The parts that decide the report are the sources list, the differential, and the chain of reasoning from facts to conclusion — not its length or its confidence.

The doctor must be cross-examined, and the appellant pays. Guide 7.6.5 is explicit: *a medical report on its own cannot be considered without the expert giving oral evidence and being available for cross-examination*. However comprehensive the report, Dr Krishnaiah must attend and be tested. Ask his rooms for a **fee estimate** and whether he will give evidence by **telephone or video** (which the Guide permits). A report the author will not stand behind in the box is not a ceiling; it is a liability.

It cannot be instructed. *A report that reads as instructed is worth less than no report.* The questions must be **neutral** — “What, if anything, do you make of the reports of fatigue recorded in April and May 2024, in the context of the diagnosis you made in October 2024?” not “Do you agree the fatigue was an early symptom?” No desired conclusion may ever be suggested; the “*perceives as unjust*” line is reasoned about, never retracted under instruction. And critically, the report **cannot lift causation above what the facts and the clinical picture actually support**. The March-to-June interval, the bereavement and relationship stressors, the ADHD/Vyvanse history are real. The report earns its weight by **confronting** them — naming them, placing them in time, reasoning through them — not by ignoring them. A report that overclaims is easier to break than one that concedes and still concludes. The ceiling is set by an honest report that reasons, not by an advocate’s document that asserts.

Why “ceiling” is the exact word

The tornado is flat: three blocks, each swinging the outcome roughly equally. But two of the three — the manner case and the discipline — are **already built and are now matters of conduct**, not of new evidence to be created. The third, causation, is the **only block still open**, and the only one that turns on a **document that does not yet exist**. Its own probability ranges from 0.62 (a weak report) to 0.91 (a strong one), and because the blocks are roughly multiplicative, that ~29-point swing on block 1 propagates through the entire result — moving the hearing from ~30% to ~55% and settlement from ~45% to ~70%.

Every other lever in the matter operates *below* this one. A sharper submission, a better witness list, a cleaner cross-examination of Ms Taylor, a further Calderbank — each helps, but none can lift the case above the causation the report supports, because causation gates the whole conjunction and because the report is the only instrument that can supply it. Nothing the appellant does after 12 August can raise the roof higher than Report B allows. That is what makes it the ceiling: **the single point at which the highest-value, least-substitutable, most-decisive work in the entire appeal is concentrated in one document — built once, on Wednesday, and never fully replaceable afterward.**

Everything routes through Wednesday. Spend the effort on the report and the discipline; the rest is done.